

Chapter 10 — Ethics and Informed Consent

- As urological practice grows more complex, its ethical and legal implications deepen.

MEDICAL ETHICS

History of Medical Ethics

- **Hippocrates** ("father of medicine"); the **Hippocratic Oath** (Box 10.1) is the cornerstone of ancient Greek medical ethics: the primary goal of medicine is to protect the patient's interests.
- **The Oath introduces** beneficence, non-maleficence, confidentiality and accountability to the profession.
- **20th-century critique (Robert Veatch)**: the Oath relies on the physician's judgement because the patient is presumed unable to understand — it **undermines patient autonomy**, and it ignores **justice** in resource allocation (treats the individual outside social context).
- **Bioethics** emerged as the Oath was criticised:
 - 1960s — debate on technology (antibiotics, genetic testing).
 - **1970 — Van Rensselaer Potter coins "bioethics"**: study of the moral relationship between humans and their social and physical world.
 - Nazi eugenic experiments → focus on autonomy and protection of medical/genetic information.
 - **Early 1970s — Tuskegee syphilis study** becomes public → reinforces autonomy and consent in research.
 - **1979 — Beauchamp and Childress: the Four-Principles framework**, still the cornerstone of modern medical ethics.

BOX 10.1 TRANSLATION OF THE ORIGINAL HIPPOCRATIC OATH (FRANCIS ADAMS, LONDON, 1849)

- **Duty to teachers**: honour them as parents; teach their sons without fee; teach only the oath-bound.
- **Beneficence / non-maleficence**: regimen for the benefit of patients; abstain from the deleterious.
- **Prohibitions**: no deadly medicine; no abortive pessary; no seduction of patients; **"I will not cut persons labouring under the stone"** — left to its practitioners (the urological line).
- **Confidentiality** of all seen or heard; **accountability** — honour for keeping the Oath, the reverse for breaking it.

The Four-Principles Framework

- **A common moral language; equal in weight, each prima facie binding** unless in conflict — conflict is the dilemma, requiring prioritisation.

COMPARISON: THE FOUR PRINCIPLES

Principle	Meaning	Grounding and consequences
Autonomy	Respect for "deliberated self-rule": patients may choose even against the physician's calculation of maximal benefit	Kantian categorical imperative (persons as ends, not means); bounded by justice (others' autonomy). Founds informed consent , confidentiality , avoidance of deceit ; depends on communication
Beneficence	Optimise benefit to the patient, viewed through the patient's values	Hippocratic duty to protect the patient's interests

Principle	Meaning	Grounding and consequences
Non-maleficence	Avoid or minimise harm	<i>Primum non nocere</i> . With beneficence: (1) be able to deliver promised benefit (training, standards); (2) be clear on probability and magnitude of risk and benefit — hence also a duty to research
Justice	Fair distribution of resources, benefits and risks	Equality ≠ justice ("people may be treated unjustly even if treated equally"). Individual: limit self-interest, respect self-determination, avoid waste. Societal: competing values — equal access, need, advocacy, cost limitation, choice

- **Pattern:** beneficence and non-maleficence are the paired Hippocratic duties; autonomy and justice are the modern additions the Oath lacked.
- **Trap:** autonomy is **not** automatically supreme — it is bounded by justice and by others' autonomy.

Medical Ethics in Clinical Practice

- **Principles do not translate cleanly to cases;** Jonsen's "**Four-Box**" technique (Table 10.1) systematises their application.

TABLE 10.1 THE FOUR-BOX METHOD TO APPLICATION OF ETHICS IN CLINICAL PRACTICE (MODIFIED FROM JONSEN, SIEGLER AND WINSLADE, 1998)

Box	Principle	Questions and content
Medical indications	Beneficence, non-maleficence	Problem, history, diagnosis, prognosis; acute/chronic/critical/emergent/reversible; goals; probability of success; plan for failure; how to benefit and avoid harm. Physician weighs risks and benefits, recommends, discloses uncertainty ; when risk-benefit is equivocal, defer to the patient's preference
Patient preferences	Autonomy	Stated preferences; informed of risks and benefits, understood, consented?; capacity and legal competence; prior preferences (advance directives); appropriate surrogate using appropriate standard; unwilling or unable to cooperate, and why; is the right to choose respected? Choice must be voluntary
Quality of life	Autonomy, beneficence, non-maleficence	Prospects of return to normal life; provider biases about the patient's quality of life; deficits if treatment succeeds; could continued life be judged undesirable by the patient; rationale to forgo treatment; comfort/palliative plans. Most ephemeral box — substituting the provider's view risks bias; elicit the patient's values
Contextual features	Justice	Family, provider, financial, religious/cultural factors; justification to breach confidentiality; resource allocation; legal implications; research or teaching; conflicts of interest; team dynamics and coordination

- **Strength:** systematic organisation of conflicting priorities for discussion with patient and family.
Weakness: may oversimplify and not resolve the decision — use with the surgeon's assessment of reasonable alternatives, seen through patient preference.
- **Trap:** quality of life draws on three principles, not one; contextual features map to **justice**.

Concept — *Medical ethics moved from the physician deciding for the patient (Hippocratic beneficence) to the patient deciding with information (autonomy), with justice added to place the individual in society. The Four-Box method simply assigns each principle a practical question set.*

INFORMED CONSENT

History of Informed Consent

- Origin in battery law, now within negligence.

COMPARISON: THE LEGAL MILESTONES

Case / year	Distinguishing principle
1905–1914 state courts	Right to bodily integrity → consent before a procedure; right to be informed of and weigh risks
Mohr v Williams, 1905	Consulting a physician does not imply consent
Luka v Lowrie, 1912	Implied consent only in emergency / life-threatening condition with an incapable patient
Schloendorff v Society of New York Hospital, 1914	Patients may limit consent: consent to examination under anaesthesia ≠ consent to surgery
Salgo v Leland Stanford Jr University, 1957	Term "informed consent" coined: disclose facts needed for an intelligent consent (risks, benefits, alternatives)
Kansas Supreme Court, 1960	Failure to inform = negligence : must show conduct outside the "reasonable and prudent" physician and proximate cause of harm
Canterbury v Spence, 1972 (DC Court of Appeals)	Disclosure standard = what a " reasonable person " (patient) would want to know, not what a reasonable physician would disclose

- **Trajectory:** battery (touching without consent) → negligence (inadequate disclosure) → patient-centred standard. **Trap:** Salgo coined the term; Canterbury set the reasonable-patient standard.

Elements of Informed Consent

- **A process** of discussing treatment, consequences, risks, benefits and alternatives; may reduce errors and empower patients.
- **Four elements:**
 1. Clinical problem, proposed treatment and alternatives, **including no treatment**.
 2. Risks and benefits compared with alternatives, and clinical **uncertainty**.
 3. **Assessment of the patient's understanding**.
 4. Solicitation of preference and consent.
- **Scope:** what could affect the decision (extraneous detail may be withheld), in lay language; **discuss options before recommending** to limit undue influence.
- **"Material risk"** has no consensus; a **1% cut-off** has been proposed, with rarer risks disclosed according to severity.
- **Limits to patient understanding:** decisional shortcuts; **optimism bias** (own risk underestimated); **framing** (survival vs death). **Counterintuitively, disclosing uncertainty improves understanding and trust.**
- **Patients idealise surgeons** and accept passively → heightened duty to invite questions. Ensure understanding of goals, nature of the procedure, outcomes and the early postoperative course.
- **Final element — explicit consent or refusal**, with implications explained; directiveness varies with how active the patient chooses to be (Fig. 10.1); ask what matters most (work, sexual function, continence, leisure).

FIG. 10.1 MODELS OF PATIENT DECISION MAKING (SCHRAGER ET AL., 2017)

Model	Clinician → patient	Patient → clinician
Paternalistic	Information and recommendations	Nothing

Model	Clinician → patient	Patient → clinician
Informed medical decision making	Information only	Nothing (patient decides alone)
Shared decision making	Information and recommendations	Values and preferences

- Only shared decision making is bidirectional.

Exceptions to the Informed Consent Requirement

- **Emergency. Consent presumed** where life or well-being is significantly threatened; disclosure waived for the unconscious patient (**Barnett v Bachrach, 1943**); parental consent waivable if the child risks "immediate injury or death".
 - **Limits:** a conscious patient may refuse even life-saving treatment (**In re Quackenbush, 1978**); no waiver for children when seeking parental consent poses no immediate harm (**Rogers v Sells, 1936**).
- **Patients without capacity.** Family empowered by law; standard is **substituted judgement** (what the patient would want), falling back to **best interests**. If the surrogate is not pursuing the patient's wishes → ethics board/administration → consider petitioning court for a **conservator**.
- **Paediatric patients.** Parents are surrogates for minors (<18 years); children participate as able. Surgeon facilitates when parent and child disagree; pre-visits to perioperative areas reduce anxiety.

Quality of Informed Consent

- **Two tests:** was enough information given, and was consent based on it? Adequacy is judged by the **reasonable-person standard**, respecting the patient's view of their interests.
- **Most consent processes are incomplete;** the **commonest missed element is assessing understanding**.
- **Forms often exceed 12th-grade reading level**, most patients read well below it, and reading the form does not improve understanding (worse with limited English).

COMPARISON: INTERVENTIONS TO IMPROVE CONSENT

Intervention	Effect
Simplified consent form alone	Mixed results
Supplemental simplified written material	Greater improvement in recall
Decision aids	Better knowledge, less decisional conflict, more participation, better risk perception
"Repeat-back" (patient explains, clinician clarifies, reassess)	Effective for comprehension but adds much time — limits feasibility

Shared Decision Making

- **Informed consent = legal route to autonomy; shared decision making (SDM) = ethical route to empowerment**, especially where several reasonable options exist.
- **Two components:** (1) information exchange — clinician gives options and pros/cons, patient gives values; (2) decision made or deferred **jointly**.
- **About 40 SDM models**, mostly for screening, testing or treatment; key components: describe options, recognise both parties' expertise, reach mutual agreement.
- **Barriers:** variable implementation, no formal training, reliance on the patient's (or perceived) informational capacity; real decisions also involve family, organisational culture and financial constraints.

Concept — Consent law protects autonomy by requiring disclosure; its weak point is comprehension, not disclosure. Every remedy — decision aids, repeat-back, SDM — shifts effort from what the surgeon says to what the patient understands.

Special Considerations

End-of-Life Decisions

- **Uncertainty** (goals, preferences, family, futility) causes variable care and overtreatment; **16% of annual Medicare spending falls in the final 2 months of life.**
- **Non-beneficial treatment:** physicians — burden outweighs benefit; patients/families — care not aligned with agreed goals. Conflict stems from uncertain benefit, differing expectations, poor prognostic communication, distrust.
- **Uniform Health-Care Decisions Act (2006)** defers to providers on whether an intervention is futile; ethically, withholding life-prolonging care at the expense of quality of life may be justified, especially if it does not alter the underlying illness.
- **Early goals-of-care discussion** with patient and support system (including palliative care, do-not-resuscitate (DNR) orders, advance directives).
- **Patient Self-Determination Act (PSDA; enacted 1990, in force 1991):** institutions must inform patients of the right to an advance directive, including refusal of life-sustaining treatment. **Fewer than half** of such preferences become DNR orders → burden falls on ill-informed families.
- **Futility policies and hospital committees** frame decisions and reduce moral distress; outcome data are limited and some groups may be disproportionately harmed.

Medical Research

- **Federal regulation (45 Code of Federal Regulations (CFR) part 46)** requires "legally effective informed consent".
- **Waiver permitted:** data not linked to identifiers; secondary research on existing data; institutional review board judges risk ≤ minimal and research impracticable without waiver.
- **General requirements:** consent before involvement, time to consider, understandable language, minimal coercion.
- **Required content:** purpose, duration, procedures, interventions, risks/discomforts, benefits, alternatives, confidentiality, contact for questions, **voluntariness without penalty**; if more than minimal risk — compensation and treatment available for injury.
- **Vulnerable populations (Subparts B-D):** children, pregnant women and fetuses, persons with disabilities, prisoners — extra requirements (e.g. **assent in children**); participation limited when risk exceeds minimal without prospect of benefit or important knowledge. Protection may reduce equitable access to research benefit.

Concept — *At the end of life and in research the same tension recurs: protection versus access. Advance planning and clear policies protect the patient's voice; over-protection of vulnerable groups can deny them benefit.*

KEY POINTS

- **Autonomy, beneficence, non-maleficence and justice** remain the foundation; informed consent is essential to surgery, with limited exceptions.
- **Research consent** is more complex, especially in vulnerable populations.
- **Consent is often incomplete**; the surgeon must actively facilitate understanding and participation.
- **End of life:** early goals of care, advance directives and defined futility policies reduce conflict.